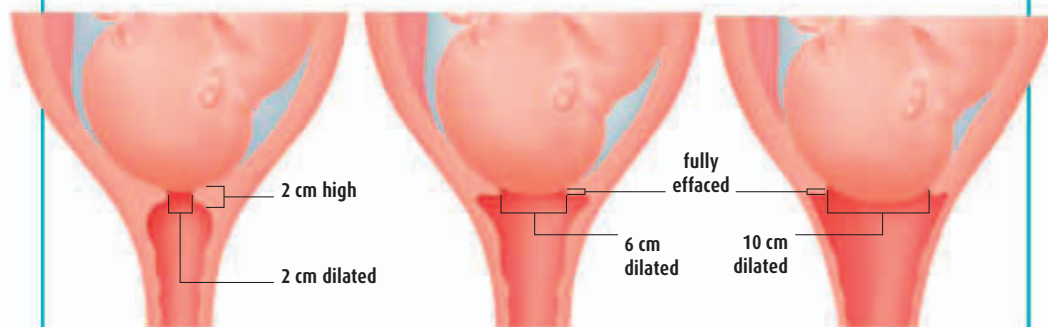


Q What does cervical dilation mean and how does it affect my labor?

In pregnancy, the cervix forms a closed, thick muscular base for the uterus. In early labor, it starts to soften, draw up, and thin (efface) and then open (dilate) so that your baby can pass into the vagina to be born.

Your doctor assesses how far and how quickly your labor is progressing by looking to see how much your cervix has thinned, or effaced, and how wide it has opened, or dilated. The width of the dilation is measured in centimeters. These changes begin in early (or latent) labor, but they can happen slowly because the contractions that act on the cervix are irregular at this point. You may feel as if your labor isn't progressing if the cervical width isn't increasing, but the cervix could still

be thinning before opening wider. Dilation can cause the mucus plug to come away from the cervix. Once the cervix has reached 3–4 cm dilation, it marks your passage from the latent to the active phase of first stage labor. In active labor, contractions become stronger, longer, and more frequent and start to open up, or dilate, the cervix more rapidly. Once the cervix is 10 cm dilated, you are fully dilated and at this point you will feel an urge to push, or bear down.



At around 2 cm dilation The cervix is just beginning to open. This is the early (or latent) part of labor and it's likely that your contractions are still fairly irregular.

At 6 cm dilation You are well established in active labor. Your contractions are now strong and regular and increasingly painful, and you may want pain relief now.

At 10 cm dilation Your cervix is fully open and, once your doctor gives you the okay, you can start to push. It may feel like contractions are coming continuously now.

Q At what point should I call my doctor?

You can call your doctor as soon as strong, painful contractions begin. Your doctor will ask you questions about your contractions and how you are feeling to assess how your labor is progressing. He or she will also gauge how you are doing from the tone of your voice and how you are responding to contractions. If your contractions are painful but irregular, and you're planning to have your baby at the hospital or in a birthing center, you will probably be encouraged to labor at home for as long as you possibly can. In this case your partner should make sure you stay well-hydrated, give you back rubs and run a warm bath to help you manage the pain. Studies show that labor progresses better when women are in

comfortable and familiar surroundings, where relaxation and rest support the release of oxytocin, which in turn encourages longer and more frequent contractions. Once your contractions are occurring around every five minutes and lasting at least 60 seconds, it is time to call the doctor again to let him know you are heading in. If you're having your baby at home, your doctor will probably ask you to call again when your contractions are closer together (say, around 10 to 15 minutes apart) and lasting between 30 and 45 seconds each. Wherever you're having your baby, call your doctor (or hospital) immediately if your water breaks, or if you have any bleeding. And call at any time if you are worried, even if you've already been told to wait. Doctors are very used to reassuring and guiding women in this way the early stage of labor.

Q What happens if my early labor is very slow or it starts and stops?

Labor is never an exact science, and what can be a relatively quick experience lasting only a few hours for one mother can take days for another. It's normal for early labor to start and stop over several days, especially if this is your first baby. The timing of your contractions, and their strength and length, are the key to knowing when you've moved from the latent phase of first stage labor to a more active phase.

Q What if my baby comes quickly and I don't have time to get to the hospital?

It is very unusual for this to happen with a first baby. If you do think labor is progressing too quickly for you to move to the hospital, try to remain calm. First call your doctor, then either your hospital emergency number or 911 for an ambulance, then your birth partner. If you are by yourself and have a friend or family member nearby, get them to come over. Gather clean towels, blankets, and a plastic bag, and choose a warm, draft-free room. If you feel the urge to push, you can try to slow things down by adopting a knee-to-chest position to take pressure off the perineum: get down on all fours, rest your forearms on the floor and raise your bottom in the air. If you still feel an urge to push, go with this. Reach down to see if you can feel the baby's head, and once your baby is born, put her immediately onto your belly for warmth, dry her with a clean towel and lay another towel and blanket over her. Leave the cord intact until help arrives. When you feel another urge to push, go with this to deliver the placenta, which you can place in a plastic bag.

Q What will happen when I arrive at the hospital?

Once you get to the hospital, you will check in to the maternity ward. Then depending on whether you appear to be in active labor, either you will be given a labor room, or you will wait in an assessment bed so that a nurse can come and check on your progress (see opposite). If your nurse assesses that you are still in early labor, you may be sent home and told when to return.